



CareLoop AI

Healthcare Commitment Intelligence & Closed-Loop Care

From a doctor's word, to a verified outcome — nothing falls through the cracks.

HACKATHON PITCH DECK

The problem: plans are written, then forgotten



Notes become disconnected to-do lists

Follow-up steps live in paragraphs of free text that no system tracks



Handoffs lose the thread

Referrals, tests, and next visits sit in separate systems with separate deadlines



Nobody closes the loop

Verification is a memory exercise — sticky notes, spreadsheets, and hope

Our solution

- Every commitment in a doctor's note becomes a tracked card — one per item, instantly
- Cards know their dependencies: if an MRI slips, the review waiting on it is flagged at risk automatically
- Evidence closes the loop: "MRI completed" verifies and closes the card — and everything downstream
- Patients see plain-language steps; doctors see the whole caseload — same records, role-filtered

Care plans get lost between the visit and the outcome



Clinical notes

A treatment list, written once, in free text



Discharge summaries

Follow-up steps, buried in paragraphs



Referrals & orders

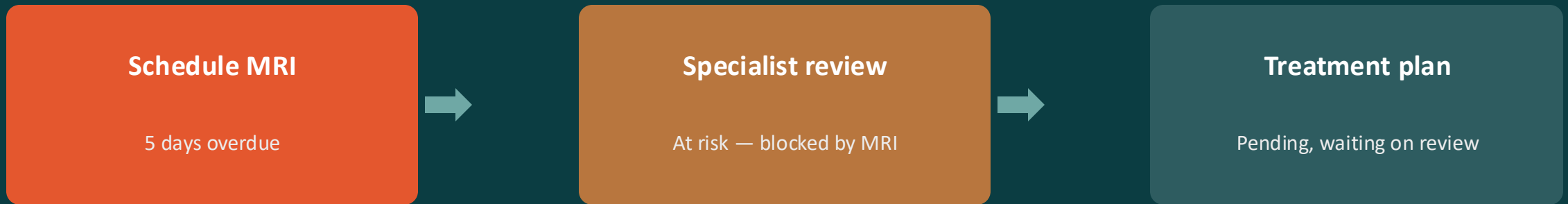
Separate systems, separate deadlines

Why it matters

- Missed follow-ups are the #1 cause of care coordination failure.
- Care coordination failures alone cost hospitals \$70K–\$220K per year just from missed referrals, duplicate tests, and coordinator burnout.
- Hospitals lose \$1–3% of Medicare reimbursement for preventable readmissions.
- Preventable readmissions cost the U.S. healthcare system \$17B annually.

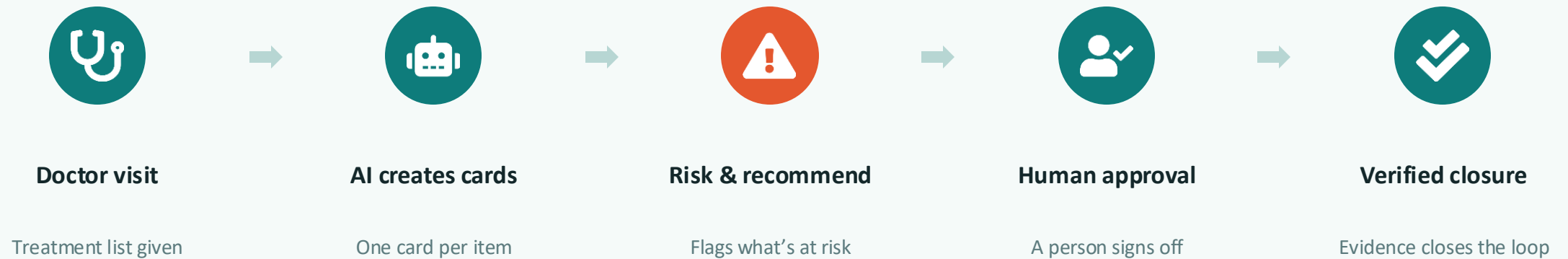
One missed step, and everything downstream is at risk

THE HEALTHCARE COMMITMENT GRAPH



Every commitment knows what it depends on — and the moment one breaks, everything downstream is flagged before it becomes a missed appointment.

From spoken word to verified outcome



The loop closes itself: once something is verified done, the graph re-checks everything that was waiting on it — no one has to remember to follow up.

The same data, two very different views



Patient login

- Sees only their own cards
- Grouped by type — pharmacy, treatment, next-visit
- Gets reminders for what's due before their next visit



Doctor / admin login

- Sees every patient's cards
- Approves flagged risks and recommendations
- Full visibility across the whole caseload

Same underlying records — filtered by role, never duplicated.

Where we are right now

Built

Note ingestion & AI card creation

Built

Commitment dependency graph & risk flags

Built

Patient & doctor/admin logins

Built

Reminders & auto-closing cards

Built

Care-gap checklists: 28 conditions wired into recommendations

Built

What's next: escalation ladder, bulk backfill, pilot hardening

Four features that make judges lean in



Missing care item detection

Flags what the doctor's note forgot to include, checked against known care protocols.



Plain-language cards

Every card doubles as a version a worried patient can actually understand.



Escalation ladder

Unanswered reminders don't repeat forever — they escalate to a human.



Bulk hospital backfill

Onboard a hospital's full existing caseload in minutes, not one patient at a time.

A background watcher that never blinks

ULTRA-FAST URGENCY FINDER



Watches continuously

Checks for new incoming files every 10 seconds — no one has to upload anything



Ultra-fast pass

A lightweight, dedicated check purely for urgency — separate from the main pipeline



Saves instantly

Flagged items are written straight to the record, ready for a doctor to see

10 sec

Maximum time before a newly arrived urgent item is caught

Runs on its own schedule, independent of the main pipeline — so an urgent item is never stuck waiting behind routine processing.

Trust isn't a feature you add later



Human approval gate

Nothing consequential happens without a person signing off first



Full audit trail

Every AI decision and human action is logged, timestamped, and traceable



Synthetic data only

No real patient data is touched at any stage of this build



Role-based access

A patient's session can never reach another patient's cards

What this is worth to a hospital

Fewer

missed follow-ups

Every commitment tracked to verified closure, not just documented

Faster

time-to-resolution

Risk is flagged the moment a dependency breaks, not weeks later

Lighter

manual chasing

Coordinators review flagged exceptions, not every single case

Verified

closures with evidence

Cards close because something proves it happened, not because someone clicked done

Simple by design



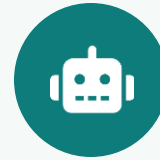
Patient & doctor apps

What people actually see and use



API layer

The rulebook every request has to pass through



AI agents

Read notes, build the graph, flag risk



Database

One shared source of truth, plain and relational

Postgres, plain and relational — no separate graph database. Dependencies are just rows linking one card to another.

From hackathon demo to hospital pilot



Hackathon MVP

Working end to end, synthetic data



Pilot readiness

Real auth, HIPAA groundwork



Hospital pilot

Real EHR data, validation study



Scale

Prior auth, referrals, payer workflows

The ROI: what the hospital gets back

Prevent

costly misses

Readmission-level failures surface while there is still time to act. **Avg. preventable readmission costs \$15,200.¹**

Reclaim

coordinator hours

The system chases every open card; people handle flagged exceptions only. **Poor care-team communication costs hospitals \$1.75M/year.²**

Shorten

time-to-treatment

Dependencies flag instantly, so the next step starts the day it can. **Inefficient handoffs waste up to 65% of admission time.³**

Prove

every outcome

Timestamped evidence on every closure — audit and quality reporting built in. **30% of malpractice claims involve a communication failure.⁴**

Source: 1. Avg. cost of a preventable U.S. hospital readmission, national cost data (AHRQ/NIH-cited, \$15,200). 2. Cost of inefficient clinical communication per hospital per year, Ponemon Institute/Imprivata Economic Impact of Inefficient Communications study. 3. Share of patient-admission time lost to inefficient communication, same Ponemon/Imprivata study. 4. Share of malpractice claims involving a communication failure, CRICO Candello national claims analysis, 2024.

What changes when the loop closes itself

1

note in

is all it takes to start a care plan — no forms, no re-entry

28

conditions

carry built-in care checklists that catch what a note forgot

0

boxes to tick

when a step is confirmed done — the evidence closes it

10s

urgency scan

The most an urgent item waits before it is flagged for a doctor to see

One platform. Many services. Your stack.

Plug in

via MCP

Any AI portal or existing app drives
CareLoop through six MCP tools

Use ours

or bring yours

The React UI ships ready for patients,
doctors, and admins

Deploy as

independent services

API :8000 · MCP :8100 ·
UrgencyWatcher — one shared
Postgres, one project

Run with

one command

docker compose up brings the
database; every service is a standalone
process



See it close a loop, live.
One doctor's note in — cards, risk,
a verified loop out.

Product cut: brag-output-2026-09-28/brag.mp4 — Healthcare Commitment Intelligence & Closed-Loop Care

Log in as dr_smith · paste a doctor's note · Process note → cards appear, risk propagates, evidence closes the loop



**Every commitment kept.
Every gap caught.
Nothing left to chance.**

CareLoop AI — Healthcare Commitment Intelligence & Closed-Loop Care

Questions?